

Response to Request for Information

HHS Health Sector AI RFI

Accelerating the Adoption and Use of Artificial Intelligence as Part of Clinical Care

VALID AI Collaborative

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Introduction

VALID AI is a community of more than 300 senior healthcare executives—including CIOs, Chief AI Officers, CMIOs, CISOs, and other health system leaders—from over 70 leading healthcare organizations committed to advancing the responsible adoption of artificial intelligence. Our members and partner organizations represent academic medical centers, integrated delivery systems, payers, community health systems, and innovation leaders.

To inform this response, VALID AI worked with its membership to identify shared barriers to adoption and convened a focused working group of member institutions and partners. The working group included leaders from Cedars-Sinai, Sutter Health, Valir Health, CTel, and the American Medical Association. The perspectives reflected here represent real-world deployment experience across diverse care settings.

Our recommendations focus on three questions:

- 1. How can AI become deflationary rather than cost-inflating?**
- 2. How can AI improve access to care?**
- 3. What can HHS and its components do to accelerate responsible and secure adoption?**

I. Making AI Deflationary

AI will reduce healthcare costs only if the cost of validating, integrating, monitoring, and reimbursing AI is lower than the value it generates. Today, much of the expense stems from duplicative validation, unclear regulatory boundaries, fragmented monitoring, and reimbursement misalignment—not model development itself. The federal government can reduce this structural friction by centralizing non-differentiated trust-building functions.

Policy Actions

1. Establish a Risk-Tiered Federal Validation Framework

- FDA, ONC, and NIH should coordinate on a dynamic, risk-based validation portfolio structured by clinical impact and AI function type.
- Rigor should scale with risk.
- Benchmarks, subgroup performance standards, and robustness protocols should be standardized and publicly available.

2. Launch Federally Coordinated AI Validation Programs

- HHS should support structured validation programs that test models against standardized criteria once at high quality and make results broadly accessible.
- FDA should align regulated product oversight; ONC should integrate outputs into health IT certification pathways.

3. Develop Representative National Reference Datasets

- NIH and NLM should steward diverse, privacy-preserving datasets reflecting rural, underserved, and heterogeneous care environments.
- ONC should align datasets with interoperability standards.

4. Create a National AI Adverse Event and Risk Signal Repository

- FDA and AHRQ should coordinate centralized signal detection for AI-related adverse events.
- ONC should ensure interoperable reporting standards.

Centralized signal detection would reduce redundant investigations and accelerate improvement cycles.

5. Align Reimbursement With AI-Enabled Value

- CMS should systematically evaluate which CPT, HCPCS, and value-based payment measures are directly affected by specific AI applications.

- CMS should partner with providers and industry to identify AI categories that measurably improve quality metrics, risk adjustment, documentation accuracy, or care management.
- The CMS Innovation Center should pilot reimbursement models recognizing AI-enabled workflow and decision support.

Without reimbursement alignment, AI risks adding administrative burden rather than reducing cost.

II. Improving Access to Care Through AI

In rural and underserved settings, the alternative to AI-supported care is often delayed or absent care. This shifts the risk-benefit calculus. AI can function as a capacity amplifier—extending expertise, reducing provider burden, and enabling earlier intervention.

Federal policy can help ensure AI expands access rather than concentrating it in well-resourced systems.

Policy Actions

1. Prioritize Rural and Underserved Demonstration Projects

- CMMI and HRSA should fund AI-enabled pilots focused on access expansion, time-to-intervention, provider burden reduction, and avoided downstream utilization.
- AHRQ should develop evaluation frameworks tailored to low-resource environments.

2. Recognize Context-Sensitive Risk Differentiation

- FDA and CMS guidance should acknowledge that acceptable AI deployment thresholds may differ where specialist access is limited, while maintaining clear safety monitoring standards.

3. Strengthen Interoperability as Access Infrastructure

- ONC should modernize API and certification requirements to reduce integration friction for AI tools.
- Standardized AI performance reporting formats should be incorporated into health IT certification pathways.

Access expansion depends on reducing deployment barriers in distributed and resource-constrained settings.

III. Accelerating Responsible Adoption Across the System

Adoption slows when organizations face regulatory ambiguity, unclear liability boundaries, and inconsistent transparency expectations. Coordinated federal guidance can reduce uncertainty without lowering standards.

Policy Actions

1. Provide Cross-Component Regulatory Clarity

- FDA, ONC, and CMS should issue coordinated guidance clarifying classification of ambient AI systems, data ingestion boundaries, and responsibility allocation between developers and providers.

2. Support AI-Enabled Transparency and Consent Mechanisms

- ONC and CMS should encourage or incentivize structured disclosure tools that inform patients when AI influences care and how oversight is maintained.

3. Reduce Procurement and Integration Friction

- HHS should explore standardized AI performance reporting templates and safe-harbor guidance for adoption of federally validated tools.

Clear rules and shared infrastructure procurement cycles and lower systemwide adoption costs.

Conclusion

AI will become deflationary when the federal government reduces the system-wide cost of creating trust. By standardizing validation expectations, supporting validation programs, building representative datasets, centralizing signal detection, aligning reimbursement, and coordinating regulatory clarity, HHS can eliminate redundant effort and lower structural barriers.

Paired with targeted support for rural and underserved deployment, this approach allows AI to serve as a capacity amplifier—expanding access while maintaining accountability.

The objective is responsible acceleration: lower friction, aligned incentives, and shared infrastructure that enables safe, scalable adoption across the healthcare system.